

To be or not to be: The identity work of pharmacists as clinicians

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Abstract

This study explores how pharmacists legitimise the expansion of their clinical work and considers its impact on pharmacists' professional identity work. In the context of pharmacy in the English NHS, there has been an ongoing policy shift towards pharmacists moving away from 'medicines supply' to patient-facing, clinical work since the 1950s. Pharmacists are continuously engaging in 'identity work' and 'boundary work' to reflect the expansion of their work, which has led to the argument that pharmacists lack a clear professional identity. Drawing insights from linguistics and specifically Van Leeuwen's 'grammar of legitimation', this study explains how the Pharmacy Integration Fund, a nationally funded learning programme, provides the discursive strategies for pharmacists to legitimise their identity work as clinicians.

KEYWORDS

identity, pharmacy, primary care, professions, workforce

INTRODUCTION

Clinical pharmacy as a form of professional practice encourages pharmacists and pharmacy support staff to shift their focus from a product-oriented to a patient-oriented approach in their

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work. The term 'clinical' refers to the focus or orientation of pharmacy activities on patients rather than medicines. Clinical pharmacy 'comprises cognitive, managerial and interpersonal activities by pharmacists regardless of setting, which target the appropriate selection, administration and monitoring of medicines by providers, individual patients and the public. Clinical pharmacy practice encompasses (but is not limited to) care models in which pharmacists assume responsibility for achieving person-centred goals for individual patients as part of a multidisciplinary team' (Dreischulte et al., 2022).

Globally, the separation of prescribing and supply functions together with the expansion of pharmaceutical industries during the 1960s meant that pharmacists no longer needed to compound medicines. In the context of pharmacy in the English NHS, since the 1950s, there has been a shift in pharmacists' work from scientifically based 'medicines supply' (a practice largely involved in making and selling medicines and dispensing prescriptions) to clinical, patient-facing work (Anderson, 2007; Elvey, 2011). The increasing availability of medicines and an ageing population living with more chronic (but treatable) conditions have led to a rise in dispensing volumes for community pharmacies. Dispensing is becoming the focus for pharmacists, and indeed the main income stream, in this setting, with community pharmacists spending less time dealing directly with patients and becoming mainly 'dispensers' (Anderson, 2007).

With ongoing changes in pharmacists' work together with their education and learning, pharmacists' professional identity is becoming less stable and enduring, and pharmacists are continuously engaging in 'identity work' (Ibarra, 1999). The expansion of pharmacists' clinical work has led to pharmacists starting to adopt identity work as clinicians, albeit only partially formed (Kellar et al., 2021). There is some perception that clinical work is more common amongst hospital pharmacists, which has been attributed previously to the clinical nature of the environment in which they work, where pharmacists were based on the wards with patients (Kellar et al., 2021).

The expansion of pharmacists' clinical work has also led to ongoing debates about the changing status of the profession (Britten, 2001; Dingwall & Wilson, 1995; Harding & Taylor, 1997; McDonald et al., 2010). This is especially the case in community pharmacy (Smith et al., 2014; The Nuffield Foundation, 1986). The nature of the work in community pharmacy and its potential contribution to health has led to the characterisation of pharmacy as an 'incomplete profession', as pharmacy lacks control over the social object of its practice (the drug) and is guided by commercial interests that are at odds with the altruistic, service orientation of professions (Denzin & Mettlin, 1968). For Dingwall and Wilson (1995), the social object of pharmacy is not the drug as a material object but drugs as the basis for social action. Hence pharmacy's professional status rests on its expertise in transforming 'chemicals' into health improving 'drugs'. A more recent and growing analysis of change in pharmacy practice has drawn on Foucault (1994) to consider how social practices and subjectivities are reconstituted in relation to changing discursive expectations. Barber (2005) suggests that the defining feature of pharmacy is its 'pharmaceutical gaze', defined as the ability to observe and predict the properties of medicines and predict their effects which no other profession has. Extending Barber's argument, Jamie suggests the need to capture the construction of the patient's body to which medicines are administered. The increased clinical and public health role of pharmacists requires pharmacists to form new ways of constructing and managing the patient's body, which is conceptualised as the 'pharmacy gaze'. Waring et al. (2016) adds that the extension of pharmacy gaze beyond the traditional pharmacological properties of the drug and its clinical health benefits, to include surveillance of how medicines interact with diverse patient health, lifestyle behaviours and adherence, requires pharmacists to have a range of non-clinical skills for questioning and guiding behaviours, which they may not necessarily have. Within the sociology of professions, at the heart of the extension of pharmacy

work is the notion of clinical autonomy, which is grounded in Freidson's (1994) characterisation of professions. Having a high degree of clinical autonomy enables individuals 'to exercise discretion in their work, to assert their own judgement and responsibility as the arbiters of their activities' (Freidson, 1994, p. 164). Advocates argue that the expansion of pharmacists' clinical work and the move from mainly being seller/supplier/dispenser (and previously medicine maker) towards clinical pharmacy may be seen as a 'professionalising movement', defined as an orientation towards pharmacy practice, which gives pharmacists greater clinical involvement as a means of enhancing their professional status (Smith & Knapp, 1972). However, in doing so, it has been argued that pharmacists are extending their work into the medical profession, which has led to a sustained debate about 'boundary encroachment' (Eaton & Webb, 1979). The other side of the argument is that the expansion of pharmacists' clinical work offers little threat of boundary encroachment. Mesler (1991), using the concept of negotiated order, argues that social power is fluid and that doctors do not need to 'lose' power for pharmacists to gain it. Moreover, the expansion of pharmacists' clinical work is accommodated within the framework of delegation, whereby medical practitioners hold the overall responsibility for patient care and informally delegate some of the clinical work to pharmacists (Eaton & Webb, 1979).

This article is situated within the sociology of professions and professional identity work. Pharmacists are expected to further expand their clinical work to include: (1) approaching interactions with patients more as consultation in a similar way that other health-care professionals would; (2) playing a key role in preventing illness, focussing on an increasing role in managing acute and chronic conditions; (3) expanding the clinical work in community pharmacy to include urgent care, medicines optimisation and safety and being the first port of call for minor illness and an agent of prevention, detection of ill health and further integration within the wider health system. To support this, the Pharmacy Integration Fund (PhIF), a nationally funded learning programme, was commissioned in 2016 to equip pharmacists with enhanced clinical and consultation skills. In this article, we used Van Leeuwen's (1996, 2007) 'grammar of legitimation' to explain how this learning programme provides discursive strategies for pharmacists to legitimise the expansion of their clinical work and identity work as clinicians.

Expansion of pharmacists' clinical work in England

Abbott (1988) argues that the best way to examine change in a profession is to investigate its work, not its organisation, and to analyse the forces that affect the content and control of that work. The expansion of pharmacists' clinical, patient-facing work started with prescribing. In the United Kingdom, pharmacists the second group of health-care professionals to become non-medical prescribers, following nurses (Department of Health, 1999). Prescribing pharmacists and nurses are not financially rewarded to the same level, nor do they acquire the same status as doctors, although they do have increased status within their own professional group (Nancarrow & Borthwick, 2005; Weiss, 2021; Weiss & Sutton, 2009). This distinction emphasises the difference, and by implication inferiority, of non-medical prescribing from medical (doctors) prescribing. However, having independent prescribing rights has been found to instill patients' confidence, acceptance and trust in pharmacists (Hindi, Seston, et al., 2019; Stewart et al., 2008). The Government's intervention in extending were pharmacist independent prescribing rights could be seen as leading to what Abbott (1988) described as competing for jurisdictional claims between pharmacy and medicines. Despite pharmacists' prescribers having more clinical autonomy over medication, new prescribers often limit themselves to their range of clinical areas/processes and

are limited by the structure of their workplace (Weiss & Sutton, 2009). Professional ideologies of pharmacy and medicine, which value indeterminacy over prescribing in mediating the outcome of competing jurisdictional claims meant that medicine has the professional status and authority over complex decision-making.

Pharmacists' work has always included a patient-facing component, traditionally focussing on advice provision and patient education. There has been an emerging role for pharmacists to be more involved with direct patient-care responsibilities. Clinical pharmacy gained prominence in England in the 1960s to reflect the nature of pharmacists' expanded roles in hospitals, moving away from the dispensary and onto wards and having more contact with patients (Cousins & Luscombe, 1995; McRobbie et al., 2018; Tyler, 1968). The development of 'clinical pharmacy' in England was akin to the development of 'patient-centred pharmaceutical care' advocated by Hepler and Strand (1990) in the United States. Unlike clinical pharmacy, which, at least in the early stages related specifically to hospital pharmacy, patient-centred pharmaceutical care is not limited to hospital pharmacy, and the focus is on pharmacists taking responsibility for outcomes of medicines use (Hepler, 1996) from only dispensing the correct medication to a greater social good by reducing drug-related morbidity and mortality (Baines, 2014).

In recent years, the expansion of pharmacists' clinical work is often described in the context of 'advanced' practice (Council for Healthcare Regulatory Excellence, 2009; Evans et al., 2021; Forsyth & Rushworth, 2021). One feature of an advanced scope of practice is the ability to use a range of clinical skills and the involvement of 'a high degree of autonomy and complex decision-making' (Health Education England, 2017). However, there is currently a lack of standardised practical clinical skills for pharmacists (Forsyth & Rushworth, 2021). Moreover, pharmacists are working in different settings, mostly in hospitals and communities and only recently in primary care; hence the ability to practice at an advanced level varies widely and is dependent on organisational support, opportunities and contractual incentives (Hann et al., 2017; Willis et al., 2019).

In community pharmacy, the 2005 NHS Community Pharmacy Contractual Framework for England (Pharmaceutical Services Negotiating Committee, 2005) introduced, for the first time, incentives and indeed mechanisms to commission medicines, clinical and public health services. However, medicine sales and supply (dispensing) remained the main source of income for community pharmacies, and most services attracted fee-per-service payments, which led to a focus on service quantity (with performance targets set) over quality, particularly by larger chains (Hann et al., 2017; Jacobs et al., 2018, 2020). The 2008 White Paper (Department of Health, 2008), which was seen as a landmark in the development of new roles for community pharmacists (Murray, 2016a), claimed that clinical services in community pharmacies, such as treatment of minor ailments (such as coughs, colds, skin problems), could deliver outcomes such as safer care and more clinical and cost-effective treatment (Watson et al., 2015). The 2014/2015 contract further expanded 'advanced services' such as Medicines Use Reviews and New Medicine Services. Although there was a widespread belief in the potential for these advanced services to contribute to professional integration and patient care, pharmacists have been unable to capitalise fully on the potential to enhance their professional status (Bradley et al., 2008; McDonald et al., 2010). This was partly due to the need for pharmacists to respond to the competitive retail pharmacy environment that prioritises income over the needs of service users and partly due to the lack the confidence to do so (Edmunds & Calnan, 2001; Hindi, Jacobs, et al., 2019; Jacobs et al., 2011; Resnik et al., 2000). Moreover, Waring et al. (2016) found that pharmacists 'pastoral role' in transforming 'drugs' into wider improvements in 'health', moving from dispensing to advice-giving to the monitoring of medicine use, patient education and intervening to change behaviours required skills that pharmacists might lack.

In general practice, the 2015 'clinical pharmacists in general practice' scheme (NHS England, 2017) aimed to recruit and train thousands of pharmacists primarily responsible for delivering clinical roles in primary care directly to patients. This scheme ended in April 2019, and clinical pharmacists are recruited under the new general practice network contract (NHS England, 2019b). Funding for learning and development support nevertheless remains, and much of this is now focussed on community pharmacy (Murray, 2016b; NHS England, 2016).

A more recent policy change in England to further expand pharmacists' clinical work goes beyond what has been implemented in the past. Under the PhIF, a nationally funded programme introduced in 2016, pharmacists are expected to approach interactions with patients more as consultations in a similar way that other health-care professionals would with a much broader perspective taken such as patient history, clinical/physical examination, diagnosis and treatment plan, which may or may not include medicine(s). Pharmacists are also expected to play a key role in preventing illnesses in the community, focussing on increasing their role in managing acute and chronic conditions, using their specialist knowledge and skills to ensure that patients get the care they need without inappropriate use of medications. In community pharmacies, the expansion of clinical work includes urgent care, medicines optimisation and safety, and community pharmacies being the first port of call for minor illness and an agent of prevention, detection of ill health and further integration with primary care by introducing the Pharmacy Quality Scheme to incentivise service quality rather than quantity/volume (NHS England, 2019a). The policy intention is to develop a sustainable supply of prescribing pharmacists with enhanced clinical and consultation skills (NHS England and NHS Improvement and Health Education England, 2020).

Pharmacists' professional identity and legitimisation

Ibarra (1999) argues that any change in work is accompanied by identity changes. People adopt 'provisional selves' defined as temporary solutions to bridge the gap between current capacities and self-conceptions and representations about new attitudes and behaviours expected in a new work role. Identity changes, therefore, 'evolve interactively such that a new synthesis is achieved that is more than simply a compromise of static role demands and static self-demands' (Ashforth & Saks, 1995, p. 173).

The ongoing expansion of pharmacists' work has consequently resulted in pharmacists' professional identity becoming less stable and enduring. Pharmacists have been found to employ multiple identities over time, encapsulating the expansion of work that pharmacists have been undergoing. Pharmacists were found to identify as 'medicine maker', 'dispenser', 'scientist', 'clinical practitioner', 'merchandiser' and 'business person' (Elvey, 2011; Kellar et al., 2020). However, as these discourses were 'piled up' rather than emerged over time, this was found to result in a weakening of pharmacists' professional identity (Kellar et al., 2020).

The diverse range of identities that pharmacists employ shows that pharmacists are continuously engaging in 'identity work' (Alvesson & Willmott, 2002; Alvesson et al., 2008; Beech, 2008; Ibarra, 1999; McDermott et al., 2013). Identity work is defined as the 'forming, repairing, maintaining, strengthening or revising the constructions that are productive of a precarious sense of coherence and distinctiveness' (Alvesson & Willmott, 2002, p. 1164). The concept of identity work implies 'agentic activity', which focuses on analysing processes of continuity and change rather than a notional end state (Brown, 2015). Hence, studies exploring pharmacists' professional identity have focussed on investigating the formation, construction, facilitation,

consolidation and socialisation of professional identity in pharmacy students (Cruess et al., 2015; Dawodu & Rutter, 2016; Jee et al., 2016; Mylrea et al., 2017; Quinn et al., 2020). Others have explored professional identity mobilisation during periods of change and uncertainty (Kreindler et al., 2012). Using a social identity approach and focussing on group level and processes rather than individual and interpersonal level (e.g. traits and skills) or operational level (e.g. structures and resources), Kreindler argued that identity mobilisation is one of the key drivers to facilitating system transformation. Identity mobilisation involves 'crafting a sense of us' that is not incongruent with the existing identity (Haslam et al., 2011). In studies exploring general practitioners (GPs) with managerial responsibilities in the English NHS, GPs have been found to employ hybrid identity work that affirmed the primacy of their identity as clinicians while incorporating managerial skills (Hotho, 2008; McDermott et al., 2013; McGivern et al., 2015). This hybrid identity was seen as more attractive than that of management (Hotho, 2008). Moreover, having a hybrid identity means that GPs are able to oscillate their identity depending on the different situations they are in (McDermott et al., 2013). In pharmacy, this means that pharmacists need to start crafting their new hybrid identity incorporating their identity work as clinicians. A recent scoping review of how pharmacists perceived their professional identity found that pharmacists have started to embrace their identity work as clinicians (Kellar et al., 2021).

Linked with an understanding of identity formation and socialisation is the concept of 'communities of practice' and 'situated learning'. In their earlier work, Lave and Wenger (1991) suggested that most of practitioner learning occurred at the workplace rather than a classroom setting, a concept known as 'situated learning' (Lave & Wenger, 1991). Newcomers becoming experts by being given opportunities to learn by engaging in simple tasks, moving from legitimate peripheral participation to full participation and acquire the identity of community members. Wenger (1998) developed the concept of communities of practice further by focussing on socialisation, learning and identity development. He argued that communities of practice are bounded by mutual engagement between individuals to create a shared meaning, community members moving towards a common goal and using common resources and jargons to negotiate meaning and facilitate learning within the group. Hence, learning is an integral aspect of social practice that involves identity construction and that identity construction occur within and across communities of practice. The use of communities of practice and situated learning to understand identity formation and socialisation has been explored in pharmacy (Hindi et al., 2022), medicine (see Cruess et al., 2018) and nursing (Woods et al., 2016).

For pharmacists, what is more important than learning to become clinicians is for their professional identity as clinicians to be perceived as 'desirable, proper, or appropriate within some socially constructed system of norms, values, beliefs, and definitions' (Suchman, 1995, p. 574). Legitimation is a process of explaining and justifying this institutional order (Berger & Luckmann, 1991). Legitimation ensures that pharmacists' professional identity as clinicians is accepted as the norm by patients, doctors, other health-care staff and pharmacists themselves. The process of legitimation can be enacted by argumentation, that is, providing arguments that explain the why of our social actions, ideas and thoughts (Reyes, 2011) and presenting the proposal as the 'right' or 'appropriate' thing to do. Silverstein (1992) referred to this as 'sociocultural legitimation'.

Legitimation has a role in professional legitimation (Klossner, 2008). Successful role performance, defined as gaining acceptance and affirmation from members of the community, is an important determinant of legitimation. Studies on professional legitimation have looked at how professionals earn and manage legitimacy (Goodrick & Reay, 2010; O'Meara et al., 2018; Reay et al., 2006; Suchman, 1995). Kellar et al. (2020) used a historical critical discourse analysis to explore how institutions construct governing discourses as a means to achieve professional legitimacy. In nursing, Goodrick and Reay (2010) identified the microprocesses that

'nurse practitioners' perform to legitimise their new professional identity in a well-established health-care system in Canada. Their discourse analysis of nursing textbooks found that nursing textbooks legitimised nurses' new identities by legitimising the past. Language was used incrementally to develop new arguments supporting a new identity without legitimising the old identity. They also found that their identification of legitimising processes was only focussed on arguments of moral legitimacy. Sanders and Harrison (2008) have shown how professionals develop discourses to claim jurisdictions and to legitimise their professional roles. The role that discourse plays in legitimising institutional change has also been highlighted elsewhere (Green et al., 2009; Phillips et al., 2004; Vaara & Tienari, 2008).

METHOD

Data collection

This study is part of a wider project evaluating the impact of learning programmes for pharmacy professionals under the PhIF introduced in March 2019 (Moss et al., 2021). The data for this article came from a qualitative study involving in-depth interviews with learners (pharmacists) (January–November 2020). Interview sampling was purposive and included learners on the learning pathways, employers, service leads and supervisors to enable inclusion of a broad range of perspectives. Recruitment of learners was supported by an invitation included with a survey that was part of the wider study. Employer recruitment was undertaken by email invitation and with the support of two national professional pharmacy organisations, who distributed and cascaded invitations throughout their members and networks. Recruitment of supervisors was supported by learning providers who distributed the email invitation amongst their networks of education and clinical supervisors supporting learners undertaking PhIF pathways.

For the purpose of this article, we are focussing on the following learning pathways:

1. Post-registration programmes (PR)—pharmacists working in a community pharmacy ($n = 20$).
2. Medicines optimisation in care homes (MOCH)—pharmacists working in and with care homes ($n = 13$). This programme aimed to deploy dedicated clinical pharmacy teams to provide care home residents with equity of access to clinical pharmacist prescribers as part of a multidisciplinary team.
3. Integrated urgent care (IUC)—pharmacists working in NHS111 or similar settings ($n = 7$). Their work involved handling medicine-related enquiries and issues, undertaking clinical assessment and treatment of minor ailments and prescribing where appropriate, prescribing for repeat prescription requests and providing self-care advice.

The interview participants had a wide range of experience, with some having registered over 30 years ago, whilst others were more recent entrants to the pharmacy profession.

Data analysis

We used Van Leeuwen's (2007) framework for analysing the way discourse constructs legitimisation for social practice, which originated from his earlier work on 'the grammar of legitimisation' (Van Leeuwen, 1996). Our analysis focuses on the grammar and linguistic strategies used by pharmacists to legitimise their clinical work and identity work as clinicians. In describing

Van Leeuwen's framework, we have listed the categories of legitimisation together with their sub-types and associated linguistic features in Table 1.

IM conducted the analysis supported by JA/AH who collected the data. Analysis was discussed at weekly paper meetings. Data analysis is presented in terms of the categories and sub-types of legitimisation.

RESULTS

Quotes used in this section are coded as follows: for example, MOCH.114 refers to the pathway (PR–post-registration pharmacist, MOCH–medicines optimisation in care home, IUC–integrated urgent care) and pseudonymised ID.

Authorisation

A subtype of authorisation is the authority of tradition whereby the 'usual' way of doing things legitimises one's action. However, this was not the case for the pharmacists we interviewed. The following interviewee legitimises pharmacists' clinical work by making a historical comparison ('the old world') with the present ('now') and the future ('where we should be'). The use of 'should be' also implies the authority of expertise that the interviewee possesses as a professional knowing the future direction of the profession:

I think in **the old world** where you didn't have, where we just had the sense that actually the pharmacist was integral to that pharmacy. **But now** actually we are more and more **patient facing** which is **where we should be** and I think it will with time change the public perception of what pharmacists can actually do.

(MOCH.114)

This pharmacist described how they are viewed by others as 'a knowledgeable person'. Knowledgeable here is described in the context of the clinical work that was learnt through the learning programme:

I think my colleagues look at me **as a knowledgeable person** because now I'm able to answer questions and **if there's somebody with a clinical condition coming and I would be able, I could give you the whole description of something because I've learnt it from the course.**

(PR.119)

Another pharmacist legitimised their patient-facing work (characterised as 'speaking to the patients or see the patients') by making an explicit reference to the 'medical and legal point of view', which was also learnt from the programme:

It [the learning programme] definitely gave me the confidence to say, well actually no, because **from a medical and legal point of view** I'm not covered to just do these things for you, **I need to speak to the patients or see the patients.** [...] just about protecting yourself as a practitioner.

(IUC.142)

TABLE 1 Summary of categories of legitimization and their linguistic strategies

Categories of legitimization	Sub-types	Linguistic strategies and examples
Authorisation—legitimation by reference to authority such as tradition, law and persons with institutional authority	Authority of tradition	By referring to the traditional practice or habit of doing things using verbal clauses such as ‘this is what we always do’
	Authority of confirmation	Using verbal clauses such as ‘because that’s what everybody else does’
	Authority of persons with institutional power	By referring to the person’s credentials or giving an endorsement, for example, ‘experienced’ pharmacists
		By referring to the authority of laws, rules and regulations, for example, This is what the policy stipulates
Moral evaluation—legitimation by reference to value systems, and it is generally hinted at rather than explicitly stated	Evaluative	Describing activities as ‘normal’, ‘natural’
	Abstraction	Referring to practices by ‘distilling from them a quality that links them to discourses of moral values’
	Analogies	Comparison with other activities
Rationalisation—legitimation by reference to the goals and uses of institutionalised social action	Instrumental rationalisation	Goal-oriented (where purposes are constructed ‘in people’ as conscious or unconscious aims and goals), for example, ‘I do x in order to do y’
		Means-oriented (where purposes are constructed ‘in action’ and the action as a means to an end), for example, ‘I achieve doing x by doing y’
		Effect orientation (where purposes are constructed by focussing on the outcome of actions hence there is an element of causation), for example, ‘I do x so that y’
	Theoretical rationalisation	By referring refers to whether the action is founded in some kind of ‘truth’ and provide explicit representations of ‘the way things are’

The PhIF learning programmes have given pharmacists the feeling of having ‘an equal footing’ to doctors:

In community pharmacy we’re fairly isolated [...] I guess in conversations with doctors, say there’s been queries over people’s medications, and before I may have been a bit apprehensive about, well maybe a bit overawed is a bit of a strong way to describe it... **overawed by the doctor’s superior knowledge in inverted commas.** Now, whilst undertaking this course, I feel as though, **I’m on an equal footing in terms of my knowledge of medicines and certain conditions.**

(PR.124)

However, this authority was embraced with precaution as clinical knowledge was still seen as belonging to the domain of medicine. Pharmacists were aware of the concerns about ‘boundary encroachment’ (Eaton & Webb, 1979). This was particularly the case for community pharmacists, with some GPs seeing community pharmacists as ‘glorified shopkeepers’:

And I still definitely think there is, what’s the word, a snobbiness almost between a GP and a community pharmacist [...] I think the problem is, with community pharmacy in, often being in a retail environment, I think, like the public **I think they just think we’re glorified shopkeepers** type of thing really. And they forget that we are actually healthcare professionals like them.

(PR.122)

Although a learning programme gave pharmacists the confidence to say what is ‘right’ or what medications ‘should’ be prescribed, that is, taking more responsibility for a patient, pharmacists were aware of boundary maintenance by some doctors:

Like I say, **because my clinical knowledge is starting to increase and deepen [...]** I’m more, instead of just saying, I don’t think this is right, give them something else. **I’m saying, I don’t think this is right,** I think **you should be giving them this, this and this.** So, I would say possibly, some GPs and prescribers [...] some get quite snotty with you doing that, because they feel like you’re undermining them a bit.

(PR.122)

Moral evaluation

Moral evaluation was used by pharmacists to legitimise increased responsibility over their clinical decisions. This pharmacist described the ability to tell doctors not only *what* is ‘right’ or ‘wrong’ in terms of medications but also *what should be* the ‘right’ thing to do (‘that’s how it needs to be’):

With the GPs it’s like I’m more confident to go and tell them that this is wrong, **that’s how it needs to be.**

(PR.119)

Similarly, the following pharmacist claimed that they were able to make professional and clinical decisions. Before the programme, pharmacists would hedge their language when giving doctors their recommendations (‘you *might* need to change this’). The programme has given

pharmacists the moral authority to say what ought to be ('this is the right thing to do', 'this is actually what needs to happen' and 'this is what you need to do'):

Yeah, absolutely because it feels like it's more of a, the conversation's on the same level as each other now rather than it being, the pharmacist going, oh well **I think you might need to change this ...** It's very much, **this is what I've done**, this is the conversation that I've had and **this is what needs to change**. So it's become more of a **this is the right thing to do**. So for instance that patient that was on the really expensive medication, it wasn't a, oh you might want to look into this. It was a, no, **this is actually what needs to happen ...** and **this is what you need to do**. ... Actually we've got **professional and clinical opinions and decisions** that can make a massive difference.

(MOCH.114)

Hence, pharmacists' work has moved from focussing only on dispensing and responding to doctors' prescribing behaviour to pharmacists being able to offer their clinical expertise, which is a step towards autonomous advanced patient-level assessment and decision-making.

Instrumental rationalisation

The instrumental rationalisation used by pharmacists we interviewed is effect orientation. Effect orientation is about purposes in order to explain why social practices exist and why they take the forms they do. Not all purposes are legitimisation. To serve as legitimations, purpose construction must have an element of moralisation. The purpose clause needs to include a moralised action, that is, having a quality that can link it to a discourse of values.

The pharmacists we interviewed described the effect as purposefulness, and the focus is on the outcome of actions. The following pharmacist started by exerting the authority of the policy ('there's a new push to get pharmacists, integrated them into roles in GP surgeries'). They then used effect orientation in the context of primary care (to help reduce workload by having more professionals in primary care) and in the context of personal career move (to get a more competitive edge, to move into general practice due to flexible time and to get more varied experience):

OK, so basically I think, are you aware, **there's a new push to get pharmacists, integrating them into roles in GP surgeries to help with reducing**, yes, to **having them, more professionals in the primary care** and, so it's just, I also figured out obviously because the pharmacy industry is saturated so it'll be worth it, getting extra qualifications to have more, **more competitive edge** and obviously **I was also thinking of moving into GP surgery role** because that's more flexible timewise and, and just seem to get more, different, **more varied experience** out of it as well.

(PR.118)

The following pharmacist also used effect orientation to legitimise the new patient-facing work. The effect described here is increased trust in pharmacists, specifically in terms of how patients are more likely to discuss their health issues with pharmacists. This pharmacist also compared their ability and confidence before and after the learning programme ('before I'd say...' and 'But I'm more confident ...because I've covered in my course'):

I feel as though they [patients] have a lot more trust in the pharmacists, ... And they, I have noticed a lot in the last year or so, they're more likely to open up about their own health issues and they'll say, oh what do you think this means and what do you think that means? ... and I refer them on whereas before I'd say, well I don't really know, because you don't want to give the wrong information. But I'm more confident to give them that information because I've covered in my course.

(PR.127)

Another example of effect orientation is the authority given by other health-care staff in pharmacists' ability to advise on medicine management issues:

But obviously then building relationships with the wider health care professionals, so like the SALT [speech and language therapist] and the dieticians and the ... nurse, I've been able to build really strong relationships with them so we can, **I can liaise with them quite readily now and get advice about patients** and obviously they can do the same for me and, and save you, **they're actually asking for advice from me as well on meds management issues** so it's helped to embed me really into primary care in my role.

(MOCH.102)

CONCLUDING DISCUSSION

Our study shows how the PhIF, a nationally funded learning programme to support a policy move towards an integrated health and care social system in England, has given pharmacists the discursive strategies to justify the expansion of their clinical work and identity work as clinicians. It does so by using authorisation, moral evaluation and rationalisation as their linguistic strategies. Following the programme, pharmacists have started embracing their identity work as clinicians and have expressed being on 'equal footing' to doctors. Pharmacists were able to morally justify not only what is right and wrong in terms of medication and to take the initiative and responsibility, rather than making a responsive recommendation where the doctor retains the overall decisions, hence exerting their increasing clinical autonomy. The pharmacists we interviewed also used the government-mandated policy to justify the purposefulness or outcome of their clinical work, which was to reduce GP workload as envisaged in the policy, to increase patient trust and to increase confidence of patients and other staff in pharmacists' clinical aptitude. As a result of this learning programme and the discursive strategies it provides to legitimise the expansion of pharmacists' clinical work and identity work as clinicians, pharmacists are moving away from traditional dispensing and technical work towards having more autonomy in advanced patient-level assessment and decision-making.

A similar development can be seen in nursing, with the development of 'nurse practitioners', that is, experienced nurses who undertake an advanced clinical nursing role (Barton, 2006; Pearson, 2011). It was argued that nurses' professional identity was 'blurring' and that nursing was going through an 'identity crisis' (Harmer, 2010). Nurse practitioners had to legitimise their new identity by cultivating opportunities for change, fitting a new role into prevailing systems and proving the value of the new role (Reay et al., 2006).

The strength of this study is in the methodology employed. The interview data were subjected to in-depth analysis focussing on the grammar and linguistic features used by those we

interviewed. This method of analysis gets behind the surface of what the interviewees say. This goes some way towards addressing the issues with interview data raised by Murphy et al. (1998), in particular their concern that respondents' accounts represent an idealised or rationalised version of their behaviour and beliefs. Moreover, most of the studies on pharmacists' professional identity pre-dates the move of pharmacists into advanced clinical practice. Hence, this study is one of the few that has explored pharmacists' professional identity following this move. However, as data collected were based on those who responded to an invitation to participate in the study, future studies should consider extending the analysis to a wider group of pharmacists.

Our study contributes to the literature on pharmacists' professional identity work and boundary work. In terms of pharmacists' professional identity work, Kellar et al. (2021) argued that if pharmacists do not identify as clinicians, any change in legislation, remuneration, education and training will not result in practice change (Kellar et al., 2020). This is in line with the argument made by Kreindler et al. (2012) on professional identity mobilisation as a key driver for system transformation. Our study shows how a nationally funded learning programme equips pharmacists to become members of communities of practice and provides discursive strategies for pharmacists to legitimise the mobilisation of their identity work as clinicians. This legitimacy necessitates continued acceptance and confidence by the pharmacists themselves, patients and other health-care professionals.

Studies exploring pharmacists' perceptions of their increasing clinical work have shown that pharmacists fear new responsibilities, lack confidence, are risk-averse and are unable to transition to become patient-centred practitioners (Rosenthal et al., 2010). However, some pharmacists have taken steps to adopt a more patient-centred orientation, and they have done so by obtaining independent prescribing qualifications (Rosenthal et al., 2010). Luetsch (2017) found that pharmacists' attitudes towards the expansion of their patient-facing work were generally positive and argued that this individual motivation needs to be supported by training and acceptance by the people they work with and serve. In pharmacy in England, previous schemes such as the introduction of 'clinical pharmacists in general practice' (NHS England, 2017) and previous experience of working with pharmacists have provided some acceptance of pharmacists' valuable contribution to primary care (Mann et al., 2018). However, pharmacists, especially community pharmacists, continue to report a lack of confidence in their clinical abilities and tend to revert to their technical dispensing tasks, where they spend most of their time (Hindi, Jacobs, et al., 2019; Weir et al., 2019). Moreover, a reduction in community pharmacy funding, complex and short-term commissioning arrangements, increasing workloads and GPs' negative perceptions of community pharmacy's potential to expand services continue to pose challenges to the implementation of patient-centred clinical services (Bradley et al., 2013, 2016; Hindi, Jacobs, et al., 2019; Jacobs et al., 2018). Hence, the extent to which pharmacists adopt their identity work as clinicians should not be assumed as universal, as this can vary by settings and organisational factors. There is a consensus that the move to a clinical community pharmacy service is no longer simply seen as desirable but needed to future proof traditional community pharmacies (Weidmann et al., 2022). Our study shows that the PhIF has provided pharmacists (including community pharmacists) with the skills and competencies required to perform clinical work.

This article has explored the development of clinical pharmacy by investigating the expansion of pharmacists' clinical work as suggested by Abbott (1988) when examining change in a profession. The expansion of pharmacists' clinical work was seen as competing for jurisdictional claims between pharmacy and medicines (Abbott, 1998) and encroaching on doctors' boundaries (Mesler, 1991). Some of pharmacists' clinical work were seen as usurping the role of doctors, such as the adoption of clinical practices like blood pressure monitoring (Edmunds & Calnan, 2001). Adamcik et al. (1986) argued that if doctors can steer pharmacists away from the most threatening

aspects of an expanded clinical role (prescribing, primary care for chronic disease patients and lab monitoring) and towards more subordinate and clearly defined areas of expertise (drug information consultation, pharmacology, adjustment to medicines), they will succeed in boundary maintenance and legitimising pharmacists' clinical role. However, with independent prescribing qualifications, pharmacists are already doing some of the 'threatening' aspects of the clinical role. Taking Gidden's (1984) structuration approach and recognising the dialectic of controls accommodating structure and processes, Mesler (1991) argued that the perception of pharmacists encroaching on doctors' role boundaries may be misplaced and that the gaining of power by pharmacists does not necessarily entail a loss of power by the other. Rather, role boundaries are 'constructed and maintained through social interactions in their contexts' (p. 326). In pharmacy in England, pharmacists retain the right to practice on the periphery of clinical medicine (Eaton & Webb, 1979). However, the ultimate responsibility for patients lies with the GPs.

Lastly, our study contributes to the sociology of professions, specifically the changing status of the profession from being an 'incomplete profession' (Denzin & Mettlin, 1968) to having a higher degree of clinical autonomy over patient care (Waring et al., 2016). Our study shows how a nationally funded learning programme can provide not only the learning needed to support the expansion of pharmacists' professional status but also discursive strategies to legitimise pharmacists' professional identity work as clinicians.

AUTHOR CONTRIBUTIONS

Imelda McDermott: Conceptualization (Lead); Formal analysis (Lead); Methodology (Lead); Writing – original draft (Lead). Jayne Astbury: Data curation (Lead); Investigation (Lead); Writing – review & editing (Supporting). Sally Jacobs: Conceptualization (Equal); Investigation (Supporting); Writing – review & editing (Supporting). Sarah Willis: Conceptualization (Equal); Investigation (Supporting); Writing – review & editing (Supporting). Ali Hindi: Investigation (Lead); Writing – review & editing (Supporting). Elizabeth Seston: Investigation (Lead); Writing – review & editing (Supporting). Ellen Schafheutle: Conceptualization (Equal); Funding acquisition (Lead); Investigation (Supporting); Supervision (Lead); Writing – review & editing (Supporting).

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DATA AVAILABILITY STATEMENT

Data available on request from the authors.

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REFERENCES

- Abbott, A. (1988). *The system of professions: An essay on the division of expert labour*. University of Chicago Press.
- Adamcik, B. A., Ransford, H. E., Oppenheimer, P. R., Brown, J. F., Eagan, P. A., & Weissman, F. G. (1986). New clinical roles for pharmacists: A study of role expansion. *Social Science & Medicine*, 23(11), 1187–1200. [https://doi.org/10.1016/0277-9536\(86\)90338-2](https://doi.org/10.1016/0277-9536(86)90338-2)
- Alvesson, M., Ashcraft, K. L., & Thomas, R. (2008). Identity matters: Reflections on the construction of identity scholarship in organization studies. *Organisation*, 15(1), 5–28. <https://doi.org/10.1177/1350508407084426>

- Alvesson, M., & Willmott, H. (2002). Identity regulation as organisational control: Producing the appropriate individual. *Journal of Management Studies*, 39(5), 619–644. <https://doi.org/10.1111/1467-6486.00305>
- Anderson, S. (2007). Community pharmacy and public health in Great Britain, 1936 to 2006: How a phoenix rose from the ashes. *Journal of Epidemiology & Community Health*, 61(10), 844–848. <https://doi.org/10.1136/jech.2006.055442>
- Ashforth, B. E., & Sacks, A. M. (1995). Work-role transitions: A longitudinal examination of the Nicholson model. *Journal of Occupational and Organisational Psychology*, 68(2), 157–175. <https://doi.org/10.1111/j.2044-8325.1995.tb00579.x>
- Baines, D. (2014). Pharmaceutical care: The blueprint for modern pharmacy. *Prescriber*, 25(13–16), 14–16. <https://doi.org/10.1002/psb.1228>
- Barber, N. (2005). The pharmaceutical gaze – The defining feature of pharmacy? *Pharmaceutical Journal*, 275, 7358–7378.
- Barton, T. D. (2006). Nurse practitioners-or advanced clinical nurses? *British Journal of Nursing*, 15(7), 370–376. <https://doi.org/10.12968/bjon.2006.15.7.20899>
- Beech, N. (2008). On the nature of dialogic identity work. *Organization*, 15(1), 51–74. <https://doi.org/10.1177/135058407084485>
- Berger, P. L., & Luckmann, T. (1991). *The social construction of reality*. Penguin Books.
- Bradley, F., Schafheutle, E., Willis, S., & Noyce, P. R. (2013). Changes to supervision in community pharmacy: Pharmacist and pharmacy support staff views. *Health and Social Care in the Community*, 21(6), 644–654. <https://doi.org/10.1111/hsc.12053>
- Bradley, F., Wagner, A. C., Elvey, R., Noyce, P. R., & Ashcroft, D. M. (2008). Determinants of the uptake of medicines use reviews (MURs) by community pharmacies in England: A multi-method study. *Health Policy*, 88, 258–268.
- Bradley, F., Willis, S. C., Noyce, P. R., & Schafheutle, E. (2016). Restructuring supervision and reconfiguration of skill mix in community pharmacy: Classification of perceived safety and risk. *Research in Social and Administrative Pharmacy*, 12(5), 733–746. <https://doi.org/10.1016/j.sapharm.2015.10.009>
- Britten, N. (2001). Prescribing and the defence of clinical autonomy. *Sociology of Health & Illness*, 23(4), 478–496. <https://doi.org/10.1111/1467-9566.00261>
- Brown, A. D. (2015). Identities and identity work in organizations. *International Journal of Management Reviews*, 17(1), 20–40. <https://doi.org/10.1111/ijmr.12035>
- Council for Healthcare Regulatory Excellence. (2009). Advanced practice: Report to the four UK Health Departments, July, 1–13.
- Cousins, H. D., & Luscombe, D. (1995). Re-engineering pharmacy practice: Forces for change and the evolution of clinical pharmacy practice. *Pharmaceutical Journal*, 255, 771–776.
- Cruess, R. L., Cruess, S. R., Boudreau, J. D., Snell, L., & Steinert, Y. (2015). A schematic representation of the professional identity formation and socialization of medical students and residents: A guide for medical educators. *Academic Medicine*, 90(6), 718–725. <https://doi.org/10.1097/acm.0000000000000700>
- Cruess, R. L., Cruess, S. R., & Steinert, Y. (2018). Medicine as a community of practice: Implications for medical education. *Academic Medicine*, 93(2), 185–191. <https://doi.org/10.1097/acm.0000000000001826>
- Dawodu, P., & Rutter, P. (2016). How do pharmacists construct, facilitate and consolidate their professional identity? *Pharmacy*, 4(3), 23. <https://doi.org/10.3390/pharmacy4030023>
- Denzin, N. K., & Mettlin, C. J. (1968). Incomplete professionalisation: The case of pharmacy. *Social Forces*, 46(3), 375–381. <https://doi.org/10.2307/2574885>
- Department of Health. (1999). *Review of prescribing, supply and administration of medicines*. The Crown Report.
- Department of Health. (2008). *Pharmacy in England: Building on strengths-delivering the future*. Department of Health.
- Dingwall, R., & Wilson, E. (1995). Is pharmacy really an “incomplete profession”. In J. A. Holstein & G. Miller (Eds.), *Perspectives on social problems* (Vol. 7, pp. 111–128). JAI.
- Dreischulte, T., van den Bemt, B., Steurbaut, S., & the European Society of Clinical Pharmacy. (2022). European Society of Clinical Pharmacy definition of the term clinical pharmacy and its relationship to pharmaceutical care. *International Journal of Clinical Pharmacy*, 44(4), 837–842. <https://doi.org/10.1007/s11096-022-01422-7>
- Eaton, G., & Webb, B. (1979). Boundary encroachment: Pharmacists in the clinical setting. *Sociology of Health & Illness*, 1(1), 69–89. <https://doi.org/10.1111/1467-9566.ep11006781>

- Edmunds, J., & Calnan, M. W. (2001). The reprofessionalisation of community pharmacy? An exploration of attitudes to extended roles for community pharmacists amongst pharmacists and General Practitioners in the United Kingdom. *Social Science & Medicine*, 53(7), 943–955. [https://doi.org/10.1016/s0277-9536\(00\)00393-2](https://doi.org/10.1016/s0277-9536(00)00393-2)
- Elvey, R. (2011). Professional identity in pharmacy. [PhD thesis, University of Manchester].
- Evans, C., Poku, B., Pearce, R., Eldridge, J., Hendrick, P., Knaggs, R., Blake, H., Yogeswaran, G., McLuskey, J., Tomczak, P., Thow, R., Harris, P., Conway, J., & Collier, R. (2021). Characterising the outcomes, impacts and implementation challenges of advanced clinical practice roles in the UK: A scoping review. *BMJ Open*, 11, 8. <https://doi.org/10.1136/bmjopen-2020-048171>
- Forsyth, P., & Rushworth, G. F. (2021). Advanced pharmacist practice: Where is the United Kingdom in pursuit of this “Brave New World”. *International Journal of Clinical Pharmacy*, 43(5), 1426–1430. <https://doi.org/10.1007/s11096-021-01276-5>
- Foucault, M. (1994). *The order of things: An archaeology of the human sciences*. Vintage Press.
- Freidson, E. (1994). *Professionalism reborn: Theory, prophecy and policy*. Polity Press, in association with Blackwell Publishers.
- Giddens, A. (1984). *The constitution of society. Outline of the theory of structuration*. University of California Press.
- Goodrick, E., & Reay, T. (2010). Florence Nightingale endures: Legitimizing a new professional role identity. *Journal of Management Studies*, 47(1), 55–84. <https://doi.org/10.1111/j.1467-6486.2009.00860.x>
- Green, S. E., Li, Y., & Nohria, N. (2009). Suspended in self-spun webs of significance: A rhetorical model of institutionalization and institutionally embedded agency. *Academy of Management Journal*, 52(1), 11–36. <https://doi.org/10.5465/amj.2009.36461725>
- Hann, M., Schafheutle, E., Bradley, F., Elvey, R., Wagner, A., Halsall, D., Hasell, K., & Jacobs, S. (2017). Organisational and extraorganisational determinants of volume of service delivery by English community pharmacies: A cross-sectional survey and secondary data analysis. *BMJ Open*, 7(10), 1–11. <https://doi.org/10.1136/bmjopen-2017-017843>
- Harding, G., & Taylor, K. (1997). Responding to change: The case of community pharmacy in Great Britain. *Sociology of Health & Illness*, 19(5), 547–560. <https://doi.org/10.1111/j.1467-9566.1997.tb00419.x>
- Harmer, V. (2010). Are nurses blurring their identity by extending or delegating roles? *British Journal of Nursing*, 19(5), 295–299. <https://doi.org/10.12968/bjon.2010.19.5.47062>
- Haslam, S. A., Reicher, S., & Platow, M. J. (2011). *The new psychology of leadership: Identity, influence, and power*. Psychology Press.
- Health Education England. (2017). Multi-professional framework for advanced clinical practice in England.
- Hepler, C. D. (1996). Pharmaceutical care. *Pharmacy World and Science*, 18(6), 233–235. <https://doi.org/10.1007/bf00735965>
- Hepler, C. D., & Strand, L. M. (1990). Opportunities and responsibilities in pharmaceutical care. *American Journal of Hospital Pharmacy*, 47(3), 533–543. <https://doi.org/10.1093/ajhp/47.3.533>
- Hindi, A. M. K., Jacobs, S., & Schafheutle, E. (2019). Solidarity or dissonance? A systematic review of pharmacist and GP views on community pharmacy services in the UK. *Health and Social Care in the Community*, 27(3), 565–598. <https://doi.org/10.1111/hsc.12618>
- Hindi, A. M. K., Seston, E. M., Bell, D., Steinke, D., Willis, S., & Schafheutle, E. (2019). Independent prescribing in primary care: A survey of patients, prescribers' and colleagues' perceptions and experiences. *Health and Social Care in the Community*, 27(4), e459–e470. <https://doi.org/10.1111/hsc.12746>
- Hindi, A. M. K., Willis, S. C. & Schafheutle, E. I. (2022) Using communities of practice as a lens for exploring experiential pharmacy learning in general practice: Are communities of practice the way forward in changing the training culture in pharmacy? *BMC Medical Education*, 22(1), 12. <https://doi.org/10.1186/s12909-021-03079-8>
- Hotho, S. (2008). Professional identity – Product of structure, product of choice: Linking changing professional identity and changing professions. *Journal of Organisational Change Management*, 21(6), 721–742. <https://doi.org/10.1108/09534810810915745>
- Ibarra, H. (1999). Provisional selves: Experimenting with image and identity in professional adaptation. *Administrative Science Quarterly*, 44(4), 764–791. <https://doi.org/10.2307/2667055>
- Jacobs, S., Ashcroft, D., & Hassell, K. (2011). Culture in community pharmacy organisations: What can we glean from the literature? *Journal of Health, Organisation and Management*, 25(4), 420–454. <https://doi.org/10.1108/14777261111155047>

- Jacobs, S., Fegan, T., Bradley, F., Halsall, D., Hann, M., & Schafheutle, E. (2018). How do organisational configuration and context influence the quantity and quality of NHS services provided by English community pharmacies? A qualitative investigation. *PLoS One*, 13(9), 1–14. <https://doi.org/10.1371/journal.pone.0204304>
- Jacobs, S., Hann, A., Bradley, F., Elvey, R., Fegan, T., Halsall, D., Hassell, K., Wagner, A., & Schafheutle, E. (2020). Organisational factors associated with safety climate, patient satisfaction and self-reported medicines adherence in community pharmacies. *Research in Social and Administrative Pharmacy*, 16(7), 895–903. <https://doi.org/10.1016/j.sapharm.2019.09.058>
- Jee, S., Schafheutle, E., & Noyce, P. (2016). Exploring the process of professional socialisation and development during pharmacy pre-registration training in England. *International Journal of Pharmacy Practice*, 24(4), 283–293. <https://doi.org/10.1111/ijpp.12250>
- Kellar, J., Paradis, E., van der Vleuten, C. P. M., Oude Egbrink, M. G. A., & Austin, Z. (2020). A Historical discourse analysis of pharmacist identity in pharmacy education. *American Journal of Pharmaceutical Education*, 84(9), 1251–1258. <https://doi.org/10.5688/ajpe7864>
- Kellar, J., Singh, L., Bradley-Ridout, G., Martimianakis, M. A., van der Vleuten, C. P. M., Oude Egbrink, M. G. A., & Austin, Z. (2021). How pharmacists perceive their professional identity: A scoping review and discursive analysis. *International Journal of Pharmacy Practice*, 29(4), 299–307. <https://doi.org/10.1093/ijpp/riab020>
- Klossner, J. (2008). Role of legitimization in the professional socialisation of second-year undergraduate athletic training students. *Journal of Athletic Training*, 43(4), 379–385. <https://doi.org/10.4085/1062-6050-43.4.379>
- Kreindler, S., Dowd, D. A., Star, N. D., & Gottschalk, T. (2012). Silos and social identity: The social identity approach as a framework for understanding and overcoming divisions in health care. *The Milbank Quarterly*, 90(2), 347–374. <https://doi.org/10.1111/j.1468-0009.2012.00666.x>
- Lave, J., & Wenger, E. (1991). *Legitimate peripheral participation in communities of practice*. Cambridge University Press.
- Luetsch, K. (2017). Attitudes and attributes of pharmacists in relation to practice change – A scoping review and discussion. *Research in Social and Administrative Pharmacy*, 13(3), 440–455. <https://doi.org/10.1016/j.sapharm.2016.06.010>
- Mann, C., Anderson, C., Avery, A. J., Waring, J., & Boyd, M. (2018) Clinical pharmacist in general practice: Pilot scheme – Independent evaluation report.
- McDermott, I., Checkland, K., Harrison, S., Snow, S., & Coleman, A. (2013). Who do we think we are? Analysing the content and form of identity work in the English National Health Service. *Journal of Health, Organisation and Management*, 27(1), 4–23. <https://doi.org/10.1108/14777261311311771>
- McDonald, R., Cheranghi-Sohi, S., Sanders, S., & Ashcroft, D. (2010). Professional status in a changing world: The case of medicines use reviews in English community pharmacy. *Social Science & Medicine*, 71(3), 451–458. <https://doi.org/10.1016/j.socscimed.2010.04.021>
- McGivern, G., Currie, G., Ferlie, E., Fitzgerald, L., & Waring, J. (2015). Hybrid manager-professional's identity work: The maintenance and hybridisation of medical professionalism in managerial contexts. *Public Administration*, 93(2), 412–432. <https://doi.org/10.1111/padm.12119>
- McRobbie, D., Webb, D., & Graham Davies, J. (2018). Clinical pharmacy practice. *Clinical Pharmacy and Therapeutics*, 1–13.
- Mesler, M. A. (1991). Boundary encroachment and task delegation: Clinical pharmacists on the medical team. *Sociology of Health & Illness*, 13(3), 310–331. <https://doi.org/10.1111/1467-9566.ep10492129>
- Moss, A., Howat, C., Fenton, C., Stearn, S., Blum, L., Malley, L., Schafheutle, E., Willis, S., Jacobs, S., Astbury, J., Seston, E., & Hindi, A. (2021). *Evaluation of the pharmacy integration fund learning pathways: Final report*. ICF.
- Murphy, E., Dingwall, R., Greatbatch, D., Parker, S., & Watson, P. (1998). Qualitative research methods in health technology assessment: A review of the literature. *Health Technology Assessment*, 2(16), 1–276. <https://doi.org/10.3310/hta2160>
- Murray, R. (2016a) Community pharmacy clinical services review.
- Murray, R. (2016b). *Community pharmacy clinical services review, independent review*. NHS England.
- Mylrea, M. F., Gupta, T., & Glass, B. (2017). Developing professional identity in undergraduate pharmacy students: A role for self-determination theory. *Pharmacy*, 5(2), 16. <https://doi.org/10.3390/pharmacy5020016>
- Nancarrow, S. A., & Borthwick, A. M. (2005). Dynamic professional boundaries in the healthcare workforce. *Sociology of Health & Illness*, 27(7), 897–919. <https://doi.org/10.1111/j.1467-9566.2005.00463.x>
- NHS England. (2016). Pharmacy integration programme.

- NHS England. (2017). General practice forward view. Clinical pharmacists in general practice.
- NHS England. (2019a). Community pharmacy contractual framework 2019–2024.
- NHS England. (2019b). Network Contract Directed Enhanced Service: Additional Roles Reimbursement Scheme Guidance.
- NHS England and NHS Improvement and Health Education England. (2020). We are the NHS: People plan for 2020/2021 – Action for us all.
- O'Meara, K., Templeton, L., & Nyunt, G. (2018). Earning professional legitimacy: Challenges faced by women, underrepresented minority, and non-tenure-track faculty. *Teachers College Record*, 121(12), 1–38. <https://doi.org/10.1177/016146811812001203>
- Pearson, H. (2011). Concepts of advanced practice: What does it mean? *British Journal of Nursing*, 20(3), 184–185. <https://doi.org/10.12968/bjon.2011.20.3.184>
- Pharmaceutical Services Negotiating Committee. (2005). Community pharmacy contractual framework.
- Phillips, N., Lawrence, T. B., & Hardy, C. (2004). Discourse and institutions. *Academy of Management Review*, 29(4), 636–652. <https://doi.org/10.5465/amr.2004.14497617>
- Quinn, G., Lucas, B., & Silcock, J. (2020). Professional identity formation in pharmacy students during an early preregistration training placement. *American Journal of Pharmaceutical Education*, 84(8), 1132–1139. <https://doi.org/10.5688/ajpe7804>
- Reay, T., Golden-Biddle, K., & Germann, K. (2006). Legitimising a new role: Small wins and microprocesses of change. *Academy of Management Journal*, 49(5), 977–998. <https://doi.org/10.5465/amj.2006.22798178>
- Resnik, D. B., Ranelli, P. L., & Resnik, S. P. (2000). The conflict between ethics and business in community pharmacy: What about patient counselling? *Journal of Business Ethics*, 28(2), 179–186. <https://doi.org/10.1023/a:1006280300427>
- Reyes, A. (2011). Strategies of legitimisation in political discourse: From words to actions. *Discourse & Society*, 22(6), 781–807. <https://doi.org/10.1177/0957926511419927>
- Rosenthal, M., Austin, Z., & Tsuyuki, R. T. (2010). Are pharmacists the ultimate barrier to pharmacy practice change? *Canadian Pharmacists Journal*, 143(1), 37–42. <https://doi.org/10.3821/1913-701x-143.1.37>
- Sanders, T., & Harrison, S. (2008). Professional legitimacy claims in the multidisciplinary workplace: The case of heart failure care. *Sociology of Health & Illness*, 30(2), 289–308. <https://doi.org/10.1111/j.1467-9566.2007.01052.x>
- Silverstein, M. (1992). The indeterminacy of contextualization: When is enough enough. In P. Auer & A. Di Luzio (Eds.), *The contextualisation of language* (pp. 55–75). John Benjamins.
- Smith, J., Picton, C., & Dayan, M. (2014). Now more than ever – Why pharmacy needs to act, Nuffield Trust.
- Smith, M. C., & Knapp, D. A. (1972). *Pharmacy, drugs and medical care*. The Williams and Wilkins Company.
- Stewart, D. C., George, J., Bond, C. M., Cunningham, I. T., Diack, H. L., & McCaig, D. J. (2008). Exploring patients' perspectives of pharmacist supplementary prescribing in Scotland. *Pharmacy World and Science*, 30(6), 892–897. <https://doi.org/10.1007/s11096-008-9248-x>
- Suchman, M. C. (1995). Managing legitimacy: Strategic and institutional approaches. *Academy of Management Review*, 20(3), 571–610. <https://doi.org/10.5465/amr.1995.9508080331>
- The Nuffield Foundation. (1986). Pharmacy: A report to the Nuffield Foundation.
- Tyler, V. E. (1968). Clinical pharmacy: The need and an evaluation of the concept. *Journal of Pharmaceutical Education*, 32, 764–771.
- Vaara, E., & Tienari, J. (2008). A discursive perspective on legitimization strategies in multinational corporations. *Academy of Management Review*, 33(4), 985–993. <https://doi.org/10.5465/amr.2008.34422019>
- Van Leeuwen, T. (1996). *The grammar of legitimation*. London College of Printing.
- Van Leeuwen, T. (2007). Legitimation in discourse and communication. *Discourse & Communication*, 1(1), 91–112. <https://doi.org/10.1177/1750481307071986>
- Waring, J., Latif, A., Boyd, M., Barber, N., & Elliott, R. (2016). Pastoral power in the community pharmacy: A Foucauldian analysis of services to promote patient adherence to new medicine use. *Social Science & Medicine*, 148, 123–130. <https://doi.org/10.1016/j.socscimed.2015.11.049>
- Watson, M. C., Ferguson, J., Barton, G. R., Maskrey, V., Blyth, A., Paudyal, V., Bond, C. M., Holland, R., Porteous, T., Sach, T. H., Wright, D., & Fielding, S. (2015). A cohort study of influences, health outcomes and costs of patients health-seeking behaviour for minor ailments from primary and emergency care settings. *BMJ Open*, 5(2), e006261. <https://doi.org/10.1136/bmjopen-2014-006261>

- Weidmann, A. E., Hoppel, M., & Deibl, S. (2022). It is the future. Clinical pharmaceutical care simply has to be a matter of course.-Community pharmacy clinical service providers and service developers' views on complex implementation factors. *Research in Social and Administrative Pharmacy*, 18(12), 4112–4123. <https://doi.org/10.1016/j.sapharm.2022.08.002>
- Weir, N. M., Newham, R., Dunlop, E., & Bennie, M. (2019). Factors influencing national implementation of innovations within community pharmacy: A systematic review applying the consolidated framework for implementation research. *Implementation Science*, 14(1), 21. <https://doi.org/10.1186/s13012-019-0867-5>
- Weiss, M. C. (2021). The rise of non-medical prescribing and medical dominance. *Research in Social and Administrative Pharmacy*, 17(3), 632–637. <https://doi.org/10.1016/j.sapharm.2020.05.015>
- Weiss, M. C., & Sutton, J. (2009). The changing nature of prescribing: Pharmacists as prescribers and challenges to medical dominance. *Sociology of Health & Illness*, 31(3), 406–421. <https://doi.org/10.1111/j.1467-9566.2008.01142.x>
- Wenger, E. (1998). *Communities of practice: Learning, meaning, and identity*. Cambridge University Press.
- Willis, S. C., Seston, E., Family, H., White, S., & Cutts, C. (2019). Extending the scope of community pharmacists' practice to patients requiring urgent care – An evaluation of a training programme using the Theoretical Domains Framework. *Health and Social Care in the Community*, 27(4), 999–1010. <https://doi.org/10.1111/hsc.12717>
- Woods, A., Cashin, A., & Stockhausen, L. (2016). Communities of practice and the construction of the professional identities of nurse educators: A review of the literature. *Nursing Education Today*, 37, 164–169. <https://doi.org/10.1016/j.nedt.2015.12.004>

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